INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Pertuzumab + trastuzumab + vinorelbine

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00526
Version 5

intravenous (IV) pertuzumab and IV trastuzumab on Day 1; vinorelbine on Days 1 and 8 of each 21-day cycle. Vinorelbine is prescribed by IV OR by mouth, not both. There is no planned Day 15 vinorelbine. Continue while beneficial and tolerated; treatment may change if one component causes toxicity.

Treatment stages

01 **Current combination**
21-day cycle

02 **Antibody-only phase, if prescribed**
21-day cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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04	Sources and further information	19

My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathlessness, chest symptoms, severe diarrhoea, dehydration, jaundice, unusual bleeding or sudden illness promptly. Do not wait for the next planned test.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

What each medicine does

Control the cancer and symptoms, with repeated review of benefit and tolerance.

HER2-TARGETED ANTIBODY

Pertuzumab

HER2-targeted antibody



Blocks HER2 pairing with other receptors

Pertuzumab binds a different area of HER2 from trastuzumab. It limits receptor pairing, so the two antibodies act together at different points on the same growth pathway.

HER2 pairing → Pertuzumab blocks pairing → Growth signals reduced

What to watch for

Keep heart-scan appointments. Report diarrhoea, new breathlessness, swelling or a treatment reaction.

HER2-TARGETED ANTIBODY

Trastuzumab

HER2-targeted antibody



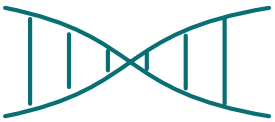
Targets HER2 growth signals

Trastuzumab binds to the HER2 protein on cells and helps reduce HER2-driven growth and recruit immune activity. HER2 testing identifies the cancers for which this approach is used.

HER2 on cell surface → Antibody binds → Growth signalling reduced

VINCA-ALKALOID CHEMOTHERAPY

Vinorelbine



Disrupts cell division

Vinorelbine interferes with the microtubules that cells use to divide. It can also affect blood cells, nerves and bowel movement. The intravenous (IV) and oral doses are different; your prescription specifies one route.

Target in the cell → Disrupts cell division → Cancer growth disrupted

What to watch for

Report new or worsening symptoms promptly and follow the agreed contact plan.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

This NCCP option is for HER2-positive metastatic or unresectable locally recurrent cancer when taxane chemotherapy is clinically unsuitable and no chemotherapy or HER2 treatment has yet been given for metastatic disease. Vinorelbine is a different chemotherapy option alongside the two HER2 antibodies. Discuss previous treatment, fitness, bowel or nerve problems and why a taxane is unsuitable.

What could change the choice?

Ask which cancer features, earlier treatments and health factors led to this recommendation, which alternatives were considered and when the decision will be reviewed.

Your treatment and the next step

Schedule

IV pertuzumab and IV trastuzumab on Day 1; vinorelbine on Days 1 and 8 of each 21-day cycle. Vinorelbine is prescribed by IV OR by mouth, not both. There is no planned Day 15 vinorelbine. Continue while beneficial and tolerated; treatment may change if one component causes toxicity.

After this phase

Continue while treatment is helping and tolerated. If a component stops, confirm the exact medicines, tests and supportive prescription for the new phase. Maintenance remains active treatment.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

The phases and how they connect

Treatment stages

01 **Current combination**
21-day cycle

02 **Antibody-only phase, if prescribed**
21-day cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

21-DAY CYCLE
Current combination

21-day cycle

IV pertuzumab and IV trastuzumab on Day 1; vinorelbine on Days 1 and 8 of each 21-day cycle. Vinorelbine is prescribed by IV OR by mouth, not both. There is no planned Day 15 vinorelbine. Continue while beneficial and tolerated; treatment may change if one component causes toxicity.

21-DAY CYCLE
Antibody-only phase, if prescribed

21-day cycle

Continue while treatment is helping and tolerated. If a component stops, confirm the exact medicines, tests and supportive prescription for the new phase. Maintenance remains active treatment.

Relative sequence, not equal lengths or booked dates. Your team confirms actual appointments and the active medicines.

BASELINE → TREATMENT → FOLLOW-UP

Tests and milestones through treatment

Columns show sequence, not equal durations or appointments. A test is only complete when performed and reviewed. Echo timing and cancer-imaging timing are different. Symptoms can bring any check forward.

Check	Before starting	First cycles	Around week 12	Continuing / next phase	Longer-term plan
Blood tests	Baseline full blood count (FBC), renal and liver profile	Before each cycle; additional dose checks as prescribed	Continue the required pre-dose checks	Use the selected active regimen's blood-test schedule	Investigate symptoms / persistent abnormalities

Check	Before starting	First cycles	Around week 12	Continuing / next phase	Longer-term plan
Echo / cardiac assessment	Baseline cardiac assessment	Earlier for symptoms / concern	Echo / multigated acquisition (heart pumping) scan (MUGA) due around 12 weeks	Continue every 12 weeks; keep the original clock	Agree follow-up for the active medicine and cardiac risk
Cancer imaging / markers	Baseline disease assessment	Clinical response; imaging if concern	Response review as planned; not tied to echo	Reassess disease and treatment plan	Imaging generally every 2–4 months; individual plan
Surgery / radiotherapy	Agree indication and sequence	No automatic concurrent radiation	Coordinate with both teams	Local treatment as indicated	Review recovery and late effects
Endocrine / bone health	Check hormone receptors / fracture risk	Plan endocrine therapy if indicated	dual-energy X-ray absorptiometry (bone density) (DEXA) when relevant to bone-risk plan	Endocrine timing by the treating team	Individual DEXA / bone follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00526 v5 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

Pertuzumab + trastuzumab + vinorelbine: the whole pathway

HER2-positive advanced breast cancer · taxanes unsuitable

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First cycles	Around week 12	Continuing / next phase	Longer-term plan
Cancer medicines NCCP	Confirm diagnosis and exact regimen	See the dose-day map: HER2 Day 1; vinorelbine Days 1 and 8	Echo due; treatment review	Review active medicines and benefit / tolerance	Review benefit / tolerance and next plan
Surgery / radiotherapy METASTATIC	Agree indication and sequence	No automatic concurrent radiation	Coordinate with both teams	Local treatment as indicated	Review recovery and late effects
Blood tests NCCP	Baseline FBC, renal and liver profile	Before each cycle; additional dose checks as prescribed	Continue the required pre-dose checks	Use the selected active regimen's blood-test schedule	Investigate symptoms / persistent abnormalities
Echo / cardiac checks NCCP · ESC	Baseline cardiac assessment	Earlier for symptoms / concern	Echo / MUGA due around 12 weeks	Continue every 12 weeks; keep the original clock	Agree follow-up for the active medicine and cardiac risk
Scans / tumour markers METASTATIC · MARKERS	Baseline disease assessment	Clinical response; imaging if concern	Response review as planned; not tied to echo	Reassess disease and treatment plan	Imaging generally every 2–4 months; individual plan
Endocrine / DEXA BONE	Check hormone receptors / fracture risk	Plan endocrine therapy if indicated	DEXA when relevant to bone-risk plan	Endocrine timing by the treating team	Individual DEXA / bone follow-up

What each check is for

1

Blood tests

FBC, kidney and liver profile at baseline and before every 21-day cycle. The team reviews the necessary results before a planned dose. Confirm the blood-count check needed before each additional vinorelbine or paclitaxel dose; a Day 1 result does not automatically authorise a later dose. For oral vinorelbine, the product information requires blood counts on the day of each administration, including Day 8; the regimen source lists full blood/renal/liver checks before each cycle.

Check safety before the next dose.

Ask / act: Agree the booking owner, result reviewer and action if overdue.

2

Heart tests

Check heart pumping function before starting HER2 treatment. Repeat echo or MUGA every 12 weeks: around cycle 5, then cycle 9 if 21-day cycles stay on time. Earlier checks are needed for symptoms or abnormal findings. Count elapsed time from the first HER2 dose; changing phase or route does not restart the clock. After the first year, ESC guidance allows six-monthly echo in selected asymptomatic low- or moderate-risk metastatic patients with normal assessments, only under an agreed team plan. Entering maintenance alone does not justify reducing checks.

Monitor pumping function and relevant rhythm risk.

Ask / **act:** Agree the booking owner, result reviewer and action if overdue.

3



Cancer imaging

Record baseline disease sites. Response imaging is generally reviewed every 2–4 months under international guidance, adapted to symptoms, disease pace and treatment. The team selects the scan; this is not an automatic computed tomography (CT) chest/abdomen/pelvis order for every patient. Tumour markers may support assessment in selected cases, but do not alone decide progression or a treatment change.

Assess response or follow-up needs.

Ask / **act:** Agree the booking owner, result reviewer and action if overdue.

4

Bone health

If the cancer is also hormone-receptor positive, endocrine treatment may form part of the wider plan. Bone-density testing is linked to aromatase inhibitors, ovarian suppression and fracture risk; it is not automatically required because a HER2 medicine is prescribed. Agree baseline DEXA where indicated and individual follow-up.

Assess fracture risk where relevant.

Ask / **act:** Agree the booking owner, result reviewer and action if overdue.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan



STEP 1

Before the first visit

Confirm the active regimen, blood results, cardiac checks, medicines review, route and consent discussion.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

The HER2 antibodies still require IV access even when vinorelbine is taken orally. Vinorelbine is given or taken on Days 1 and 8; the loading-antibody visit takes longer. A Day 8 plan may include blood review and tablets rather than an infusion, as arranged by your team.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

First visits can take longer because of loading doses, premedicines and observation. Ask the unit about expected time, transport and any escort needed after sedation or line insertion.

Phase-specific visit pattern

Infusion and observation times differ by drug and first versus later doses; the unit confirms the full visit duration, including blood review and preparation.

If taking capsules

Oral vinorelbine is taken only on the prescribed Days 1 and 8, with some food. Swallow capsules whole with water; do not chew, suck, open or dissolve them. Do not use a damaged capsule. Do not take an extra dose after vomiting; contact the team. Dose increases require a new prescription and blood-count review.

When scans, heart checks and bloods happen

Agree who books each test, who reviews the result and what happens if a check is overdue.



Blood tests

FBC, kidney and liver profile at baseline and before every 21-day cycle. The team reviews the necessary results before a planned dose. Confirm the blood-count check needed before each additional vinorelbine or paclitaxel dose; a Day 1 result does not automatically authorise a later dose. For oral vinorelbine, the product information requires blood counts on the day of each administration, including Day 8; the regimen source lists full blood/renal/liver checks before each cycle.



Heart ultrasound and electrocardiogram (ECG)

Check heart pumping function before starting HER2 treatment. Repeat echo or MUGA every 12 weeks: around cycle 5, then cycle 9 if 21-day cycles stay on time. Earlier checks are needed for symptoms or abnormal findings. Count elapsed time from the first HER2 dose; changing phase or route does not restart the clock. After the first year, ESC guidance allows six-monthly echo in selected asymptomatic low- or moderate-risk metastatic patients with normal assessments, only under an agreed team plan. Entering maintenance alone does not justify reducing checks.



Scans and tumour markers

Record baseline disease sites. Response imaging is generally reviewed every 2–4 months under international guidance, adapted to symptoms, disease pace and treatment. The team selects the scan; this is not an automatic CT chest/abdomen/pelvis order for every patient. Tumour markers may support assessment in selected cases, but do not alone decide progression or a treatment change.

Bone health and endocrine treatment

If the cancer is also hormone-receptor positive, endocrine treatment may form part of the wider plan. Bone-density testing is linked to aromatase inhibitors, ovarian suppression and fracture risk; it is not automatically required because a HER2 medicine is prescribed. Agree baseline DEXA where indicated and individual follow-up.

How surgery and radiotherapy fit

Surgery or radiotherapy may be used for particular symptoms or disease sites. There is no routine surgery-then-radiation sequence for this advanced-cancer regimen. The oncology and radiation teams agree any treatment pause or overlap.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
HER2 antibodies · Day 1	 **Day 1** IV antibodies
Vinorelbine · Day 1	 **Day 1** IV OR oral route
Vinorelbine · Day 8	 **Day 8** Only if the team confirms
Bowel movement	 **Days 1–21** Report constipation early
Blood counts / infection	 **Days 1–21** Fever or sudden illness: call

Planned treatment: IV pertuzumab and IV trastuzumab on Day 1; vinorelbine on Days 1 and 8 of each 21-day cycle. Vinorelbine is prescribed by IV OR by mouth, not both. There is no planned Day 15 vinorelbine. Continue while beneficial and tolerated; treatment may change if one component causes toxicity.

Next phase: Continue while treatment is helping and tolerated. If a component stops, confirm the exact medicines, tests and supportive prescription for the new phase. Maintenance remains active treatment.

Heart checks: Check heart pumping function before starting HER2 treatment. Repeat echo or MUGA every 12 weeks: around cycle 5, then cycle 9 if 21-day cycles stay on time. Earlier checks are needed for symptoms or abnormal findings. Count elapsed time from the first HER2 dose; changing phase or route does not restart the clock. After the first year, ESC guidance allows six-monthly echo in selected asymptomatic low- or moderate-risk metastatic patients with normal assessments, only under an agreed team plan. Entering maintenance alone does not justify reducing checks.

Why these bands stay broad: Treatment markers show the planned schedule. Symptom bands are broad possibilities, not measured risk curves; serious symptoms can occur at any time. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00526 v5 · official regimen ↗

There are no guaranteed "safe days". Serious symptoms need assessment on any day, even if the chart shows a usual recovery period.

Between-treatment notes

Day 1–21: The current treatment cycle

IV pertuzumab and IV trastuzumab on Day 1; vinorelbine on Days 1 and 8 of each 21-day cycle. Vinorelbine is prescribed by IV OR by mouth, not both. There is no planned Day 15 vinorelbine. Continue while beneficial and tolerated; treatment may change if one component causes toxicity. Your team confirms treatment dates and checks. Serious symptoms need advice on any day.

Support around the medicines

1 Route-specific support

Oral vinorelbine needs a separate nausea plan; do not copy an IV-only minimal-risk template. Confirm anti-sickness treatment, constipation prevention and blood checks before each prescribed dose.

2 Keep heart checks on the plan

An echo every 12 weeks is separate from blood tests or cancer imaging. Ask who books and reviews it.

3 Plan for symptoms

Keep your agreed contact numbers and instructions. New symptoms can need assessment before a routine appointment.

IV or oral vinorelbine: know your route

The two routes use different doses. Your team chooses one; never substitute milligram for milligram.

IV vinorelbine

Staff give this through a checked IV line. Tell them immediately about pain, burning or swelling. It must not be self-injected.

A bowel plan

Vinorelbine may slow bowel movement. Discuss prevention, any prescribed laxative and when to call; severe pain with vomiting or no stool/wind needs urgent assessment.

Questions worth asking

Who arranges my tests?

Ask the team who books each test, who checks the result and who to contact if an appointment is overdue. This map does not book appointments.

What if I become unwell between visits?

Use the urgent contact plan for sudden illness, fever, chest symptoms, severe diarrhoea or poor fluid intake. Do not wait for a scheduled blood test or scan.

Is vinorelbine a daily tablet?

No. This regimen uses vinorelbine on Days 1 and 8 of a 21-day cycle only, whether IV or oral. Oral vinorelbine is taken only on the prescribed Days 1 and 8, with some food. Swallow capsules whole with water; do not chew, suck, open or dissolve them. Do not use a damaged capsule. Do not take an extra dose after vomiting; contact the team. Dose increases require a new prescription and blood-count review.

Try explaining the plan back: Which medicines are active now, which days do you receive or take them, and who is booking and reviewing your next tests?

Pertuzumab

- Heart pumping-function reduction
- Diarrhoea and rash
- Fatigue

Vinorelbine

- Low blood counts and infection
- Constipation or bowel obstruction
- Numbness or weakness
- Nausea; oral doses need a specific plan
- Irritation if an IV dose leaks

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

intravenous (IV) vinorelbine · Days 1 and 8

Active medicines: Pertuzumab + Trastuzumab + IV vinorelbine (HER2 treatment on Day 1 only, if still prescribed)

Emetogenic risk: Minimal · IV route

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- If HER2 therapy has stopped, the IV or oral vinorelbine plan may still apply, but remove the stopped antibodies from the prescription. Confirm the active components at every visit.
- Confirm Days 1 and 8 only, active vinorelbine route, current counts and the actual prescribed dose. IV and oral doses are not interchangeable.
- Assess constipation; agree prophylactic laxative or dietary support where appropriate. Severe pain, vomiting or no stool/wind requires urgent review.
- Antibody premedication is usually unnecessary unless indicated by previous hypersensitivity; consider paracetamol/antihistamine per local policy.
- No routine IV taxane steroid or carboplatin antiemetic course applies. An individual rescue plan may still be needed.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Oral vinorelbine · Days 1 and 8

Active medicines: Pertuzumab + Trastuzumab + Oral vinorelbine (HER2 treatment on Day 1 only, if still prescribed)

Emetogenic risk: Oral moderate–high · NCCP oral classification

Medicine	Dose / route	When
Ondansetron (one 5-HT3 option)	8–16 mg total orally per day	On each prescribed oral vinorelbine treatment day; exact dose, timing and duration to be completed Choose ONE 5-HT3 regimen. Review QT, hepatic dosing, constipation and interactions; granisetron is an alternative, not an add-on.

Additional phase-specific support

- If HER2 therapy has stopped, the intravenous (IV) or oral vinorelbine plan may still apply, but remove the stopped antibodies from the prescription. Confirm the active components at every visit.
- Confirm Days 1 and 8 only, active vinorelbine route, current counts and the actual prescribed dose. IV and oral doses are not interchangeable.
- Assess constipation; agree prophylactic laxative or dietary support where appropriate. Severe pain, vomiting or no stool/wind requires urgent review.
- Antibody premedication is usually unnecessary unless indicated by previous hypersensitivity; consider paracetamol/antihistamine per local policy.
- NCCP oral-agent guidance recommends a 5-HT3 antagonist: ondansetron 8–16 mg total daily OR granisetron 1–2 mg total daily. Confirm the intermittent Day 1/8 prescription; do not automatically give a daily 21-day course.
- Do not copy the minimal IV vinorelbine classification to the oral route. Do not automatically copy a high-risk IV NK1/steroid schedule.
- Swallow capsules whole with water and some food. No extra dose after vomiting; contact the team.

- Source distinction: NCCP regimen supportive section labels vinorelbine minimal without separating routes; NCCP2025 nausea/vomiting classification Table8 places oral vinorelbine in moderate–high and Table5 gives oral prophylaxis.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

HER2-only continuation · if prescribed

Active medicines: Pertuzumab + Trastuzumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Confirm which medicine stopped and the selected continuation regimen. Remove vinorelbine-related prophylaxis if no longer needed.
- Continue cardiac monitoring; review symptoms and the active-regimen blood schedule.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

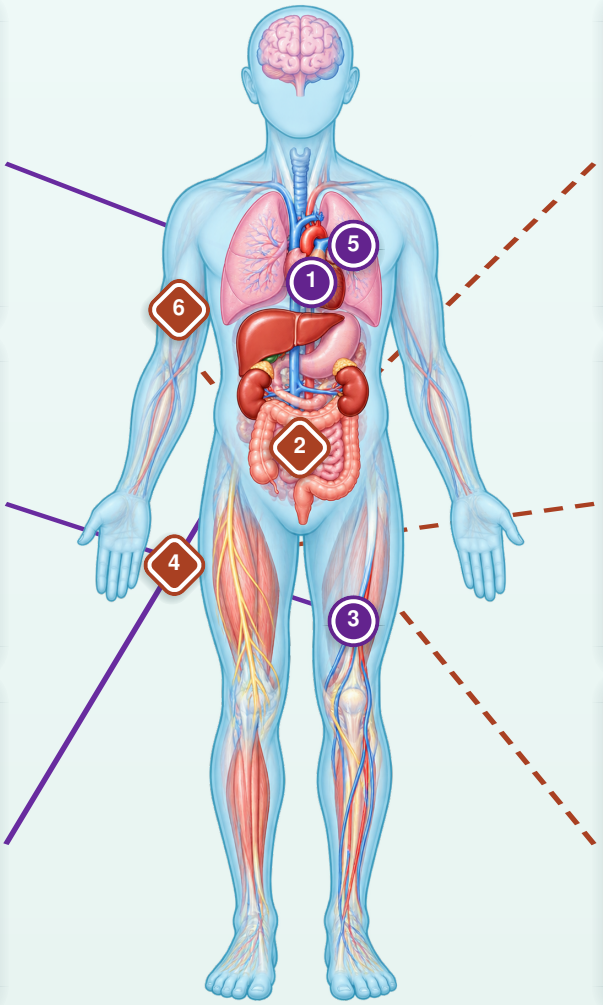
Pertuzumab + trastuzumab + vinorelbine

intravenous (IV) pertuzumab and IV trastuzumab on Day 1; vinorelbine on Days 1 and 8 of each 21-day cycle. Vinorelbine is prescribed by IV OR by mouth, not both. There is no planned Day 15 vinorelbine. Continue while beneficial and tolerated; treatment may change if one component causes toxicity.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Heart / breathing

HER2 antibodies

Keep echoes; report breathlessness, swelling or chest symptoms.

2 Constipation / bowel pain

Vinorelbine

Report constipation early; severe pain, vomiting or inability to pass stool or wind needs urgent advice.

3 Blood / infection

Vinorelbine

Fever, shivers or sudden illness: contact oncology immediately.

4 Nerves

Vinorelbine

Report tingling, weakness or difficulty with usual tasks.

5 Infusion reaction

HER2 antibodies

Tell staff immediately about rash, chest tightness or breathing difficulty.

6 IV site / oral nausea

Vinorelbine

IV burning or swelling needs immediate attention. Oral nausea needs a route-specific plan.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

The map shows selected effects of the medicines named here. Report persistent effects from earlier treatment too.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00526 v5 · official regimen**
   https://healthservice.hse.ie/documents/6630/526_v5_Pertuzumab_Vinorelbine_trastuzumab_zOGbzKp.pdf
2. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
3. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
4. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
5. **ESMO metastatic breast cancer guideline · 2026**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
6. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
7. **Irish Navelbine oral product information**
   https://medicines.ie/medicines/navelbine-r-20-mg-soft-capsule~A0000682/spc
8. **EMA Herceptin product information**
   https://www.ema.europa.eu/en/documents/product-information/herceptin-epar-product-information_en.pdf
9. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
10. **eviQ · peripherally inserted central catheter (PICC) patient information**
    https://www.eviq.org.au/p/543
11. **HSE · chemotherapy side effects and urgent advice**
    https://www2.hse.ie/conditions/chemotherapy/side-effects/
12. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
13. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
14. **EMA Perjeta product information**
    https://www.ema.europa.eu/en/documents/product-information/perjeta-epar-product-information_en.pdf
15. **NCCP systemic anticancer therapy (SACT) nausea / vomiting guidance v6 (2025) · oral treatment**
    https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf